glucoflow
RESULT ACCOUNTABILITY LEDGER

The result nobody owns.

The lab is done when it delivers. We give every report an owner, a clock and a receipt.

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01 THE PROBLEM

A lab report arrives on WhatsApp, as a PDF or as a paper slip. The clinic is supposed to assign a named clinician, set a clock, confirm the clinician saw it and confirm the patient was told. In most hospitals there is no proof that any of this happened.

- In an Indian tertiary hospital, 64.61% of 1,187 critical values were notified 30 to 120 minutes after the sample arrived. Outpatient notification was a weak point. (PMC4310845)
- A systematic review found 6.8% to 62% of abnormal ambulatory lab results were not followed up. (J Gen Intern Med 2012)
- NABL 112A and NABH already require a critical-alert register with identifiers, time, person informed and read-back. Most clinics keep it on paper or not at all.

02 THE PRODUCT

The receipt, not another record.

When a report arrives, the ledger creates one administrative receipt. The lab flag is copied from the report, never calculated.

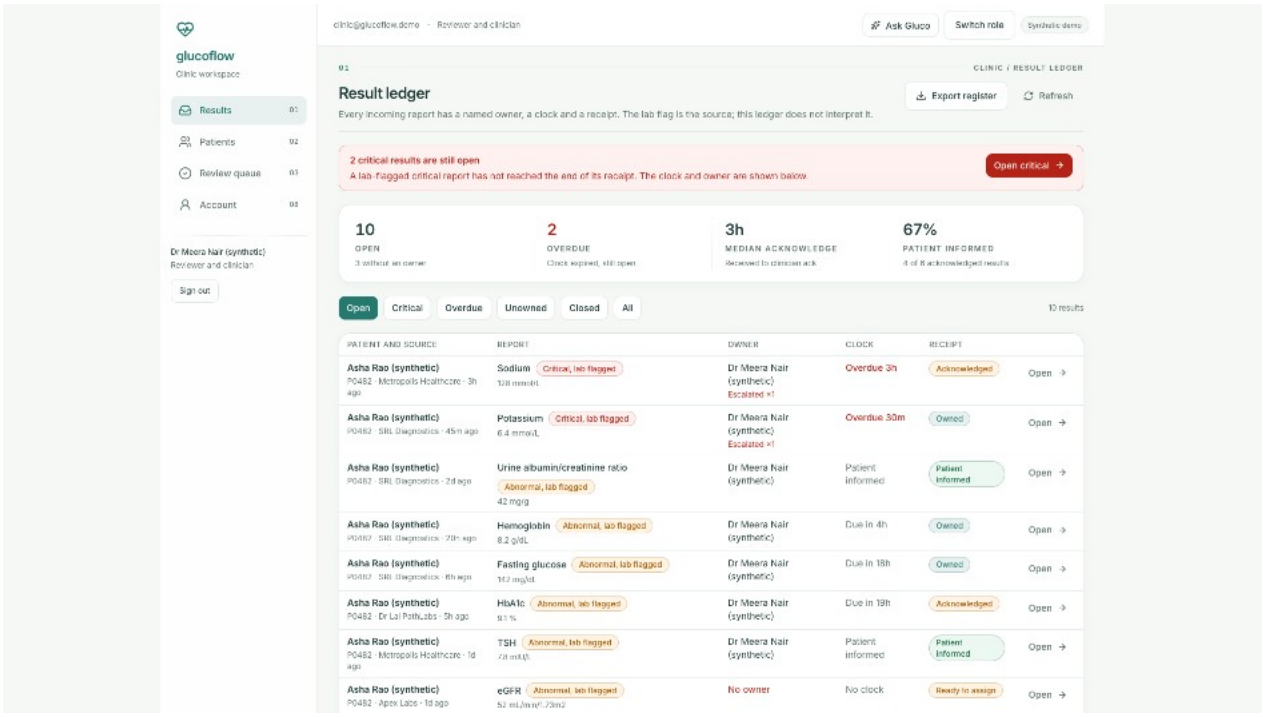
01 Ingest	02 Match	03 Own	04 Prove
WhatsApp, PDF, paper or portal. The source and lab flag are recorded.	Namesake, duplicate or wrong identifier stops for a human.	A named clinician gets a due clock and an escalation path.	Acknowledge, inform the patient, read back critical results, close.

03 THE PROOF

A receipt you can verify, not a dashboard you have to trust.

Every event is hash-chained. The receipt code opens a public verifier that checks the chain without exposing a patient name, test value or clinical decision. The hospital can download a survey pack with the evidence map. The lab can prove delivery. The patient can see that the loop closed.

RECEIPT	CHAIN	EVENTS	CONTROLS
GLF-XXXXXXXXXX	Verified	4	4 of 4



The live result ledger. Critical results, owners, clocks and the proof panel. The demo adds a synthetic inbound report with one button.

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WHY THIS IS DIFFERENT

The receipt network.

An EMR stores a result. A receipt proves the loop closed. A network makes the receipt portable.

04 THE MOAT

The receipt. A hash-chained event for every owner, clock, acknowledgement and patient communication.

The proof. A public code verifies the chain with no patient data. The survey pack maps the evidence to NABL and NABH controls.

The network. One receipt standard that labs and hospitals can share. A lab that sends 1,000 reports a month can prove delivery; a hospital can prove acknowledgement.

05 MARKET AND BUYER

The buyer is the quality head, not the doctor alone.

- Beachhead: a 50 to 200 bed private hospital or nursing home with NABH aspiration, or a district hospital quality team.
- Buyer: quality head, medical superintendent or lab director. The doctor is the user whose acknowledgement is the receipt.
- Channel: a lab partner, a district quality programme, then Tier 2 and Tier 3 hospitals.
- Price hypothesis: ₹5,000 to ₹20,000 per facility per month. No TAM claim until a paid pilot.

06 REGULATION AND AI

Administrative, not a medical device.

The clinic is the data fiduciary; the product is the processor. DPDP consent, notice, retention and audit are designed in. CDSCO software guidance keeps scheduling, communication and record transport outside the device lane. The product does not diagnose, triage, score risk or recommend treatment. The only AI job is extraction and patient matching; the lab flag is the source of criticality.

07 THE GATES

Gate	Measure	Kill rule
Baseline	100 consecutive reports at 3 sites	<30% abnormal results lack a receipt
Concierge	2 weeks, 2 sites, founder-free week	Median ack does not improve by 25%
Close	Critical read-back and patient informed	Unowned results stay >10%
Pay	Paid continuation after 8 weeks	No site signs or pays

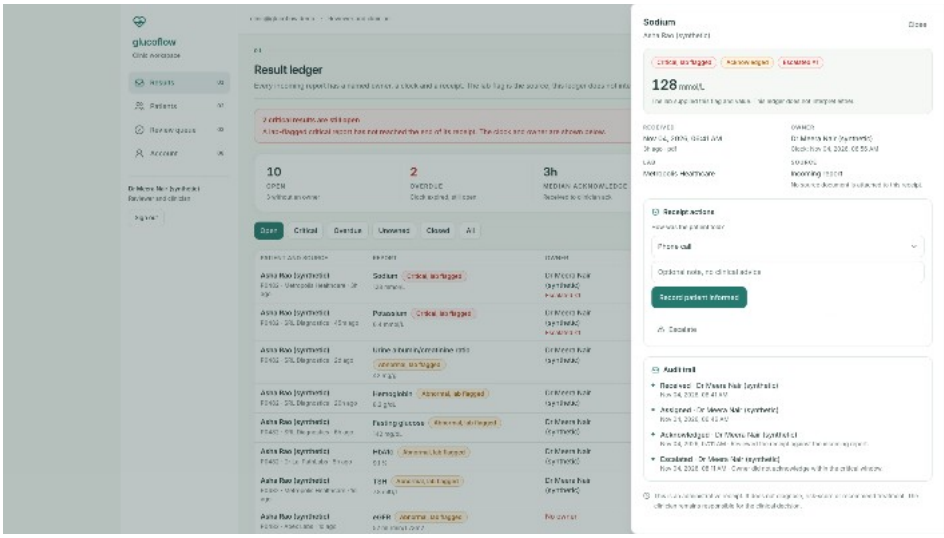
Targets, not measured results. The pilot starts with a baseline audit; if the first two gates fail, the thesis stops.

08 THE ASK

Three sites. 100 reports each. Eight weeks.

A baseline audit, not a demo. One paid continuation is the proof that the buyer exists.

The result nobody owns, until someone signs.



The receipt drawer. A named owner, a clock, the patient communication, the read-back and an append-only audit trail. The proof panel downloads the survey pack.

Synthetic data. Administrative only. No clinical interpretation. This brief is a product and validation document, not legal or clinical advice.